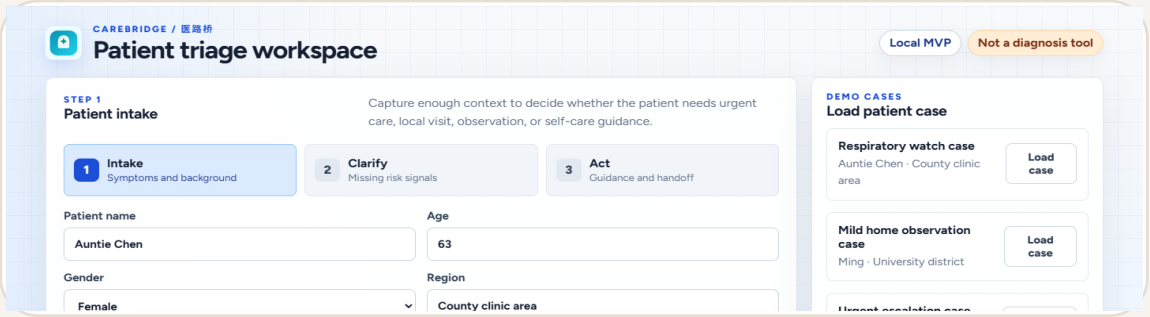
CAREBRIDGE / 医路桥

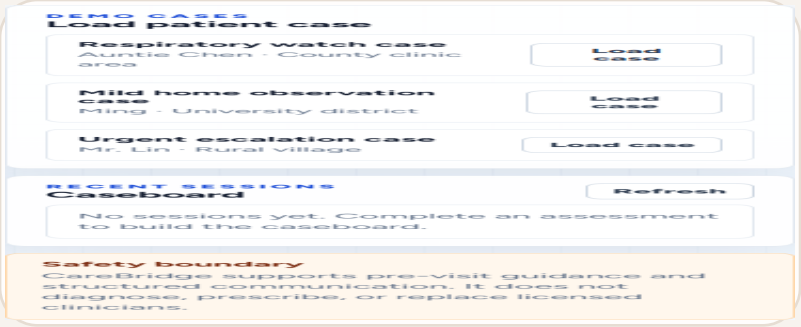
Turn symptom chaos into safer care decisions.

AI Triage & Visit Preparation Assistant for Underserved Patients
让患者在见到医生之前，就完成更清晰、更安全的就医准备。



Why this submission feels credible

- Red-flag aware triage decisions
- Follow-up questions only when missing context changes urgency
- Doctor-facing summary that can be copied, printed, or downloaded
- Persistent revisit flow for follow-up



Healthcare accessibility

Safety boundaries

Hackathon-ready

PROBLEM

The hardest part often happens before diagnosis.

The care journey breaks while patients are still deciding what to do.

Patients hesitate. Families guess. Doctors reconstruct.

Medical friction is not only about limited resources. It is also about uncertainty, delay, and fragmented communication before a patient reaches formal care.

PAIN POINT

Urgency is unclear

Users often cannot tell when symptoms have crossed the line from inconvenient to dangerous.

Who the product is built for

One narrow workflow, but value across the care chain.

1 Patients in rural and county settings

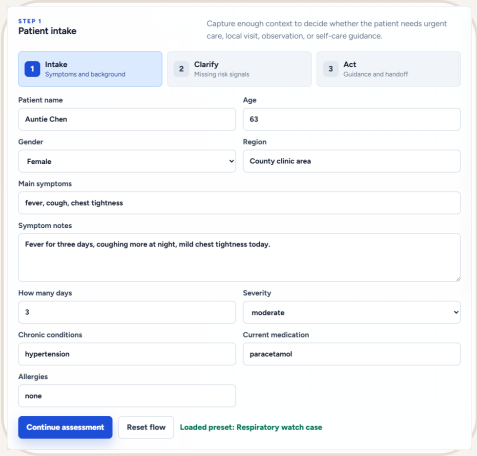
Need a clear answer to: should I go now, where should I go, and what should I say?

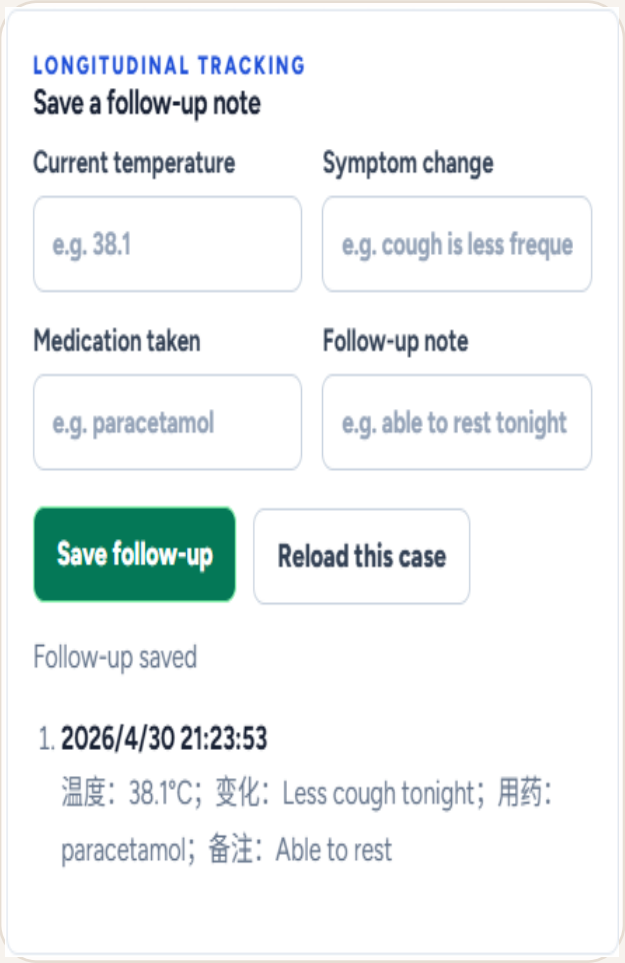
2 Family caregivers

Need a practical way to record symptoms and support elderly or low-tech patients.

3 Clinicians

Need structured context before time gets spent reconstructing the patient's story.





SOLUTION

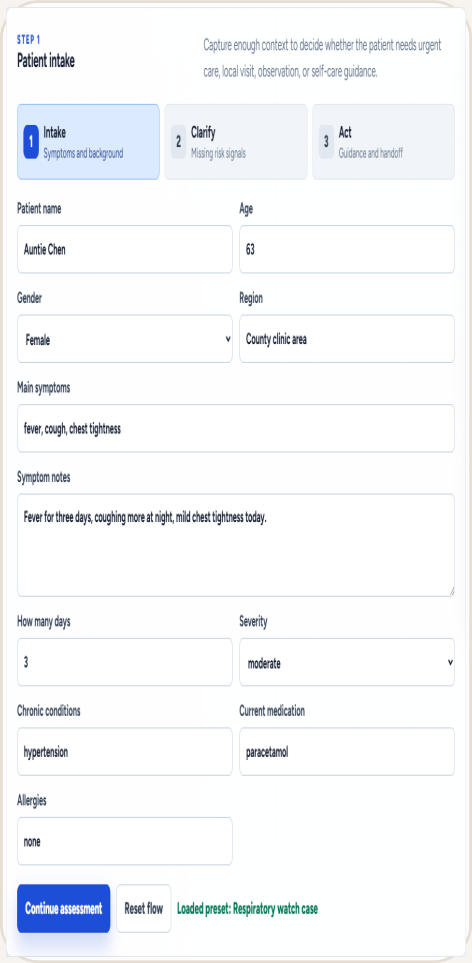
CareBridge prepares patients better

We do not replace doctors. We improve the first step into care.

A simple product promise:

Help patients move from symptom uncertainty into a safer next action.

- Start with natural language
- Ask only the missing questions that change urgency
- Return a bounded action recommendation
- Generate a summary the doctor can use immediately



WORKFLOW

How the product works

A narrow, realistic loop is easier to trust and easier to demo.

1

Intake

Symptoms + patient context

2

Clarify

Only the missing risk signals

3

Assess

Risk level and action

4

Summarize

Doctor-facing handoff

5

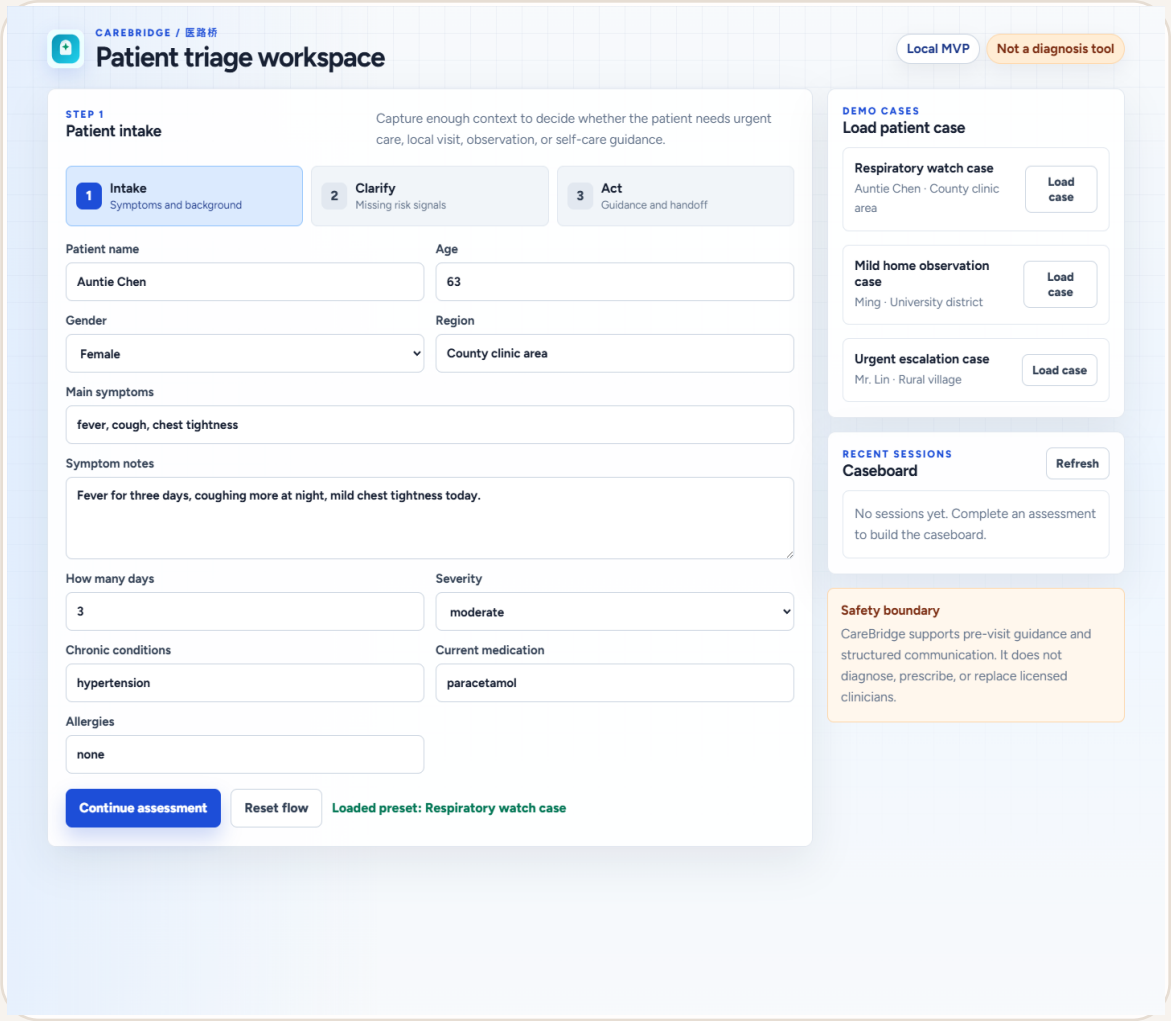
Track

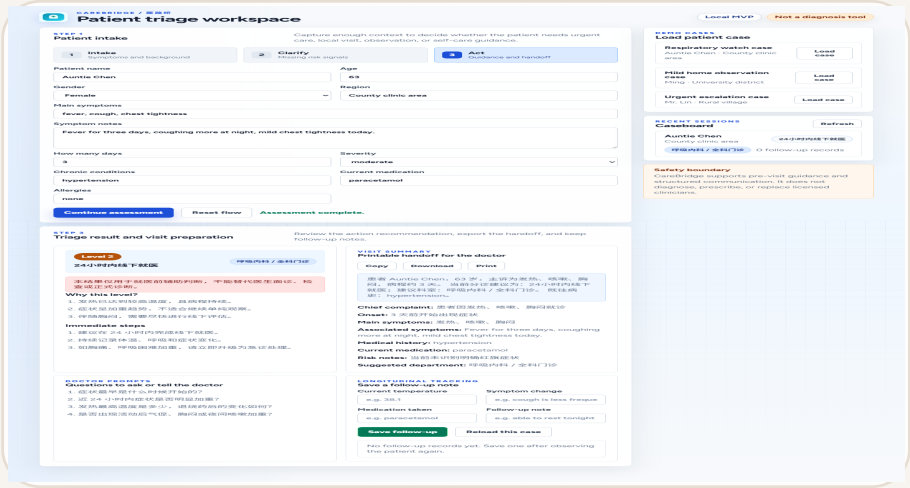
Follow-up over time

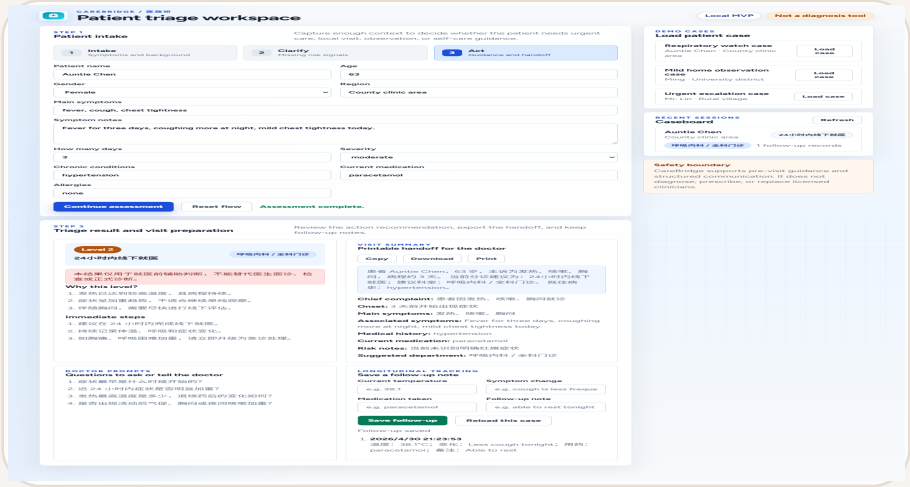
Designed to explain itself in under one minute of viewing.

From intake to output

Real screenshots carry more trust than abstract mockups.







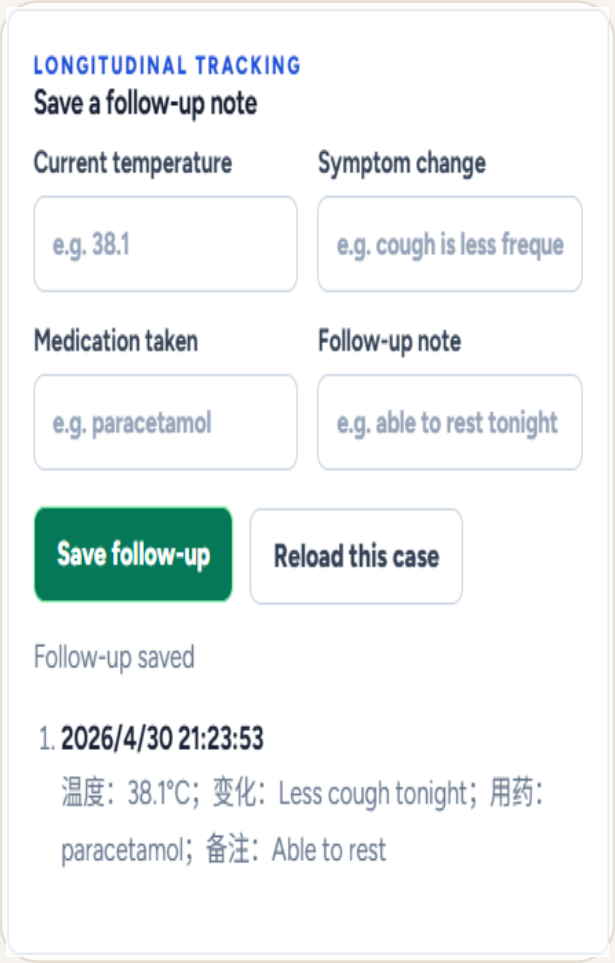
Safety boundaries built in

A good healthcare hackathon project should know its boundaries.

Not a diagnosis engine.

A better first step into the healthcare system.

- No final medical diagnosis
- No replacement for licensed clinicians
- Conservative escalation when red flags appear
- Local session storage in the MVP



EXECUTION

Built and verified

A working repo, a tested flow, and clean submission assets.

AUTOMATED TESTS

10

Unit + API tests cover triage rules and session workflows.

E2E JOURNEYS

1

Playwright validates the main patient experience end-to-end.

SCREENSHOTS

3

README, PDF, and submission pages already have polished product visuals.

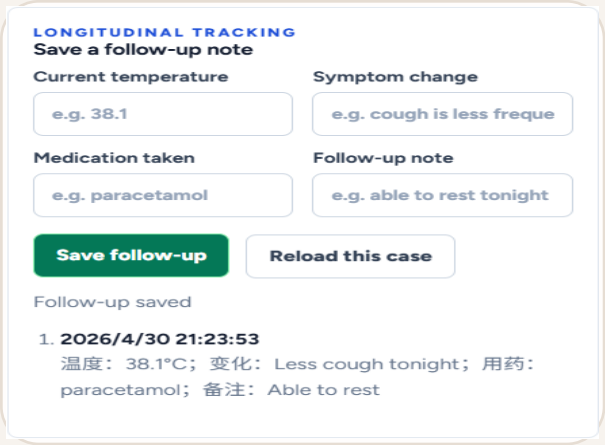
REPOSITORY

Ready

Docs, scripts, tests, and assets are packaged for submission.

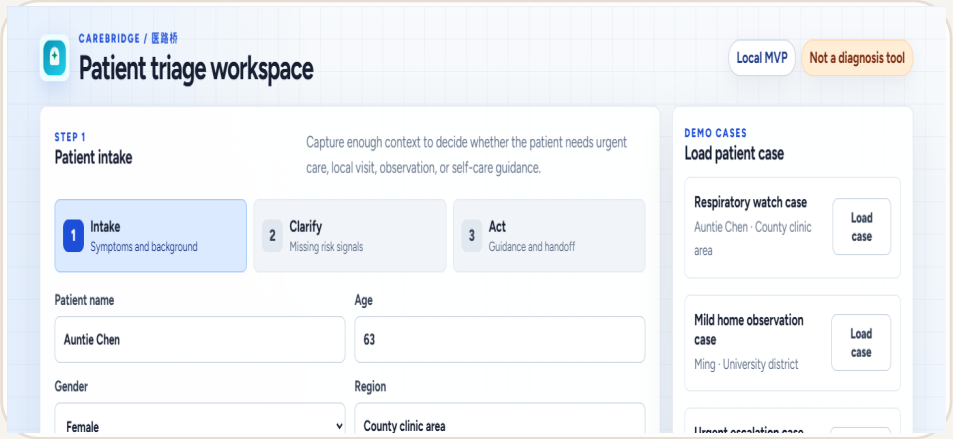
Why this matters for judges

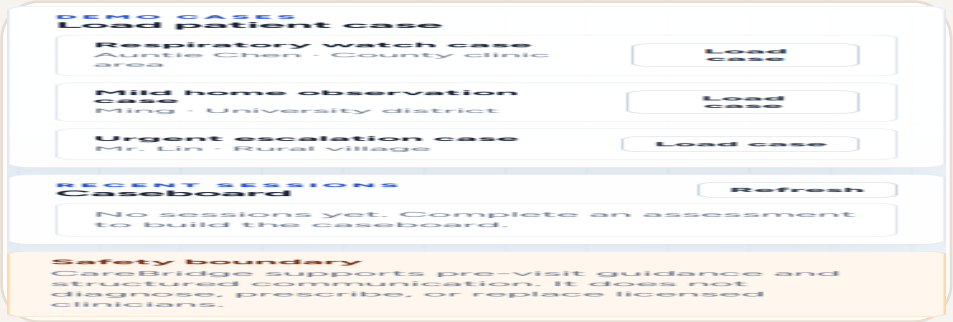
- This is a working local product, not only a concept.
- The flow is coherent enough to demo in under five minutes.
- The team thought about responsibility, not only features.



Helping patients reach care earlier, safer, and better prepared.

Not a diagnosis engine.
A better first step into the healthcare system.





Healthcare accessibility

Responsible AI

Hackathon-ready MVP